

--SUMMARY--

Decision No. 1293/21

13-Oct-2021

R.Salisbury - M.Kesler - S.Roth

- Disablement (nature of work)
- Significant contribution (of compensable accident to development of condition)
- Tear (rotator cuff)
- Tendonitis (shoulder)

No Summary Available

18 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w4921n]
- MEDICAL DISCUSSION PAPERS CONSIDERED: Shoulder Injury and Disability

Style of Cause:

Neutral Citation: 2021 ONWSIAT 1638



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1293/21

BEFORE:

R.E. Salisbury: Vice-Chair
M. Kesler: Member Representative of Employers
S. Roth: Member Representative of Workers

HEARING:

September 2, 2021 at Toronto
Oral by Teleconference

DATE OF DECISION:

October 13, 2021

NEUTRAL CITATION:

2021 ONWSIAT 1638

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) dated
September 18, 2020

APPEARANCES:

For the worker: A. Armstrong, Office of the Worker Adviser

For the employer: M. Mitchell, Paralegal

Interpreter: Not applicable

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REASONS

(i) Introduction

- [1] The worker appeals a decision of the ARO dated September 18, 2020. As the final decision of the Workplace Safety and Insurance Board (the “Board” or “WSIB”), the ARO concluded that the worker does not have initial entitlement to tendinosis of the right supraspinatus tendon under a claim in January 2015 for a gradual onset disablement injury. The ARO also concluded that the worker does not have entitlement under a claim in respect of the May 28, 2015 compensable workplace accident for injuries identified in an MRI dated November 10, 2015 which showed an almost full thickness tear of the worker’s right supraspinatus tendon. The ARO rendered a decision based upon the written record without an oral hearing.

(ii) Issues

- [2] The issues under appeal are as follows:
1. Initial entitlement to tendinosis of the right shoulder supraspinatus tendon claimed in January 2015 to be the result of a gradual onset disablement.
 2. Entitlement under a claim in respect of the May 28, 2015 compensable workplace accident for injuries identified in the November 10, 2015 MRI diagnostic imaging of the worker’s right shoulder which identified an almost full thickness tear of the worker’s right supraspinatus tendon.

(iii) Background

- [3] The following are the basic facts.

(a) Supraspinatus Tendinosis

- [4] The employer, an automobile manufacturer, reported on an Employer’s Report of Injury (“Form 7”) that on January 22, 2015 the worker reported a gradual onset injury to his right shoulder claimed to be caused by repetition. The worker reported that he became aware of this injury on January 10, 2015. At the time of the reported onset, the worker was 38 years of age and had been employed full time for approximately 2 ½ years as a maintenance team member with the employer. The employer recorded the worker as reporting:

...my work involves a lot of physical activities, and stress, strain to my shoulders. I don’t have a particular incident which would have caused this injury. Something I caught on during the period of [employer name] Maintenance activities.

- [5] A Triage Assessment report dated January 16, 2015 from a general hospital emergency department local to the worker indicates that the worker sought medical attention on January 16, 2015 for a reported complaint of:

...right shoulder pain X approximately 3 months, no specific injury, used to just hurt with strain/exertion but now constant, dec [decreased] ROM [range of motion], reports swelling not visualized, no numbness/tingling to right arm, NAD

- [6] The worker was discharged from the emergency department on January 16, 2015 with a discharge diagnosis of right shoulder strain and instructions to engage in physio and follow up with his family doctor.
- [7] On referral from the worker's family doctor Dr. S. George, the worker underwent right shoulder diagnostic ultrasound imaging on March 10, 2015. Dr. C. Coblenz, radiologist, reported that the worker's right shoulder images indicated tendinosis of the supraspinatus tendon, but there was no evidence of a tear.
- [8] An Eligibility Adjudicator ("EA") with the Board contacted the worker on March 27, 2015 by telephone. In a memorandum of that same date, the EA recorded the worker as reporting, in part:
- Worker states that he has pain in r shoulder since the beginning of the year – was using pain relief balm. It didn't go away so he decided to see/saw a dr.
 - Went for physio, got better but didn't get all the way better
 - Still going to physio centre in [name of city]
 - He is a maintenance worker- duties vary a lot every day – he attributes the pain to his regular duties – there was no particular moment/incident that caused the pain
 - He feels that his work is repetitive even though his tasks vary all the time
 - He's r handed
 - Reg duties: many physical activities, tightening/loosening of bolts, working on milling machines/lathes, climbing on top of machines, using screw driver, using cordless drills, pushing/pulling/lifting of heavy metal parts is the biggest reason for the pain (IW believes), using heavy torque wrench to torque heavy bolts is the 2nd biggest reason for the pain
 - On avg 10 times a week he's pushing/pulling/lifting heavy carts
 - He uses heavy torque wrench 1-2 a week avg.
 - There was no change in job duties in the past 3 years – there was no increase in workload/hrs – every day is different and he does different duties throughout the day
 - He works 8 hrs/day
 - He's been doing this job for about 3 years
 - Prior to this he was a maintenance person at a different location – he's been doing maintenance for about 14 years
 - He told his employer about his pain 22Jan15 (approx.) – he doesn't remember exactly – he spoke with [name of supervisor] – he said he was having pain in his shoulder and he was getting tx [treatment] – IW [worker] didn't want to make a claim at that point as he was hoping it would go away
 - First saw a dr. on 16Jan15 or 17Jan15 (he doesn't remember which), he went to [name of local hospital] emergency – he had x-ray done at the hosp that same day – nothing wrong with bones – gave him anti-inflammatores and recommended physio – he started physio on 19Jan15 and he did it for 2 weeks
 - Physio said that if it's a sprain, it should go away in 2 weeks – if it doesn't go away, he should see his family dr for mri or u/s [ultrasound]

- He then saw fam dr on 15Feb15 or 16Feb15 – Dr. Susan George- she gave him a different anti-inflammatory and booked an u/s– this was done on 18mar15 – the results show no visible tear, dx [diagnosis] is tendinosis of supraspinatus tendon
- He is still doing physio, he will be going on 30Mar15 – he will be doing physio for about 6 wks
- He does not have any follow ups booked w/Dr. George unless pain gets worse
- He's still on reg duties but he's not doing anything that will harm his shoulder
- Had r shoulder pain 6-7 years ago – a sprain- and it went away after 6-8 weeks – he hasn't had any ongoing shoulder issues until now
- He wasn't seeing a dr. on a reg basis for shoulder prior to Jan15- he was not on restrictions at work prior to Jan15

...

[9]

Following the conversation with the worker on March 27, 2015, the EA denied the worker's January 2015 claim for right supraspinatus tendinosis as a result of a gradual onset disablement. In describing the reasons for that decision in a memorandum dated March 27, 2015, the EA wrote, in part:

- Entitlement has not been granted in this case. The worker reports a gradual onset of r shoulder pain that began at the beginning of Jan15. He has been a maintenance worker for the past 14 years and with [employer name] for the past 3 years. His regular duties involve fixing and maintaining machines and lathes. His work is varying and dynamic and his duties change from day to day. He attributes his r shoulder pain to pushing/pulling/lifting heavy parts on average of 10 times per week and using a heavy torque wrench an average of 1-2 times per week. He confirms that his regular duties have not changed in the past 3 years and there has been no increase in workload. He had an MRI done and has been diagnosed with tendinosis of r supraspinatus tendon.
- As his regular work duties are varied and dynamic and there has been no change in his work duties in the past 3 years I cannot find a causal link between his work duties and the onset of r shoulder pain that occurred at the beginning of Jan15.
- Furthermore supraspinatus tendinosis is a degenerative disease that occurs over a long period of time. This condition is due to a number of factors such as age, genetic predisposition, and potentially other outside activities, and is not just due to work activities. As a result, this diagnosis is not compatible with the accident history reported.

[10]

In advancing the worker's objection to the EA's March 27, 2015 decision through to an appeal before the ARO, the worker's representative secured a medical opinion dated February 28, 2019 from Dr. P. Mathew, orthopaedic surgeon. With respect to the worker's claim for entitlement to right supraspinatus tendinosis as a result of a gradual onset disablement, Dr. Mathew advised:

I have been involved in [the worker's] care from July 6, 2015 to August 7, 2018 inclusive. I had furthermore performed an arthroscopic rotator cuff on [the worker] on October 4, 2016.

In response to your questions asked of me, I have provided my opinion as follows:

1. [The worker's] duties as a Maintenance/Millwright involved significant pushing, pulling, lifting and overhead work. I would strongly opine that this type of

repetitive task would be a significant contributing factor in [the worker's] shoulder problems. This was reported by Dr. C. Coblenz, MD, FRCPC based on the ultrasound performed by the ultrasound technologist as "Tendinosis."

...

2. I strongly believe that the work performed by [the worker] as a Maintenance Worker involving overhead work, climbing ladders etc. had a direct causal relationship to the medical condition of [the worker's] right shoulder as seen in the MRI's of June 1 and October 11, 2015 but first defined in the ultrasound of the March 10, 2015 as tendinosis of the supraspinatus tendon.

[11] The ARO in denying the worker's entitlement to right supraspinatus tendinosis as a result of a gradual onset disablement, and thereby confirming the original March 27, 2015 decision of the EA, found that:

Based on the worker's statement and the PDA, I do not find evidence to support the worker's regular job duties in general involve risk factors for a shoulder injury. Instead, I find no evidence of significant pushing, pulling, lifting or overhead work...

...Dr. Mathew's opinion that the worker's diagnosis of tendinosis is compatible with the worker's regular job duties is based on the worker's job involving significant pushing, pulling, lifting and overhead work.

As I do not find the evidence supports the worker's regular job duties involve these activities, I have not placed significant weight on Dr. Mathew's finding that the job duties are a significant contributing factor in the worker's shoulder problems.

(b) Findings of the November 10, 2015 MRI

[12] The employer on a Form 7 dated June 22, 2015 advised the Board that on May 28, 2015 the worker reported:

"climbing ladder, put my body weight on my R shoulder pulled shoulder, believe sprained-strained."

[13] Dr. George, as the worker's family physician, advised the Board on a Health Professional's Report ("Form 8") dated June 15, 2015 that on May 28, 2015, the worker pulled his right shoulder while climbing a ladder. Dr. George diagnosed the worker as having right rotator cuff tendonitis.

[14] In correspondence to the Board dated September 29, 2016 the worker described the May 28, 2015 accident as follows:

I was climbing a vertical ladder (straight up 90 degrees with ladder built by steel round piping to access the material hopper with a check sheet in my left hand). I was hopping from one step to the other just using my right hand (I pull my body with my right hand and simultaneously I move up a step). I did not slip or trip but twisted my body and I felt a pull inside my shoulder in one of my hops and I have reported this and clearly mentioned it in the accident report.

[15] An MRI without contrast (dye) of the worker's right shoulder on June 1, 2015, four days after the May 28, 2015 accident, was reported by Dr. D. Ho, radiologist, as indicating marked rotator cuff tendinosis in the worker's right supraspinatus tendon, however a discrete tear was not observed.

[16] In a memorandum dated June 26, 2015 an EA confirmed that the worker's claim for strain tendonitis was allowed, but the worker's claim for right shoulder tendinosis was denied. The EA's review commentary stated:

The accident history in this claim is the worker was climbing a ladder at some point the worker had his full weight being carried by the right arm causing shoulder pain.

The diagnostics note tendinosis – this is not caused by this type of single event

The form 8 notes tendonitis and sprain strain – this would be in keeping with the acc history.

I have tried contacting the worker however the workers [sic] does not accept incoming calls.

[17] The worker remained on modified duties with the employer and continued with physiotherapy after his May 28, 2015 compensable accident. The worker was first seen by Dr. P. Mathew, orthopaedic surgeon, on July 6, 2015 at a clinic operated on behalf of the employer. Dr. Mathew noted that the MRI of the worker's shoulder without contrast in June 2015 revealed only some tendinopathy in the supraspinatus tendon. Dr. Mathew stated that his plan for the worker at that time was:

I elected to inject the subcoracoid process area as well as the glenohumeral joint with a combination of Depo-Medrol and Xylocaine. After the injection, I retested him, and he was able to move his shoulder normally. I suspect that he does have a case of tendinopathy. Hopefully, this will help him. I have asked to see him again in about a month or two...

[18] The worker was referred by the Board to its Shoulder and Elbow Program for assessment by Dr. K. Faber, orthopaedic surgeon. In an Early Shoulder Physician Specialist Assessment Report dated July 31, 2015, the worker's January 2015 onset of right shoulder pain was described by Dr. Faber as:

Jan. right sh RC (uncertain what started pain as it came on gradually) – resolved in a couple of months

In that same report the worker's mechanism of injury on May 28, 2015 for his compensable right shoulder injury was described as:

Pulling up workers body weight while climbing a ladder. Worker was climbing a ladder when pulling himself up with his right arm he had most of his body weight through his right arm when he reports 9/10 pain.

The report went on to provide that the worker should:

1. Continue with work restrictions as above for 4-6 weeks. Restrictions can be reevaluated by physiotherapist or family doctor and progressed as symptoms improve.
2. Continue with physiotherapy treatment for frequency of 2-3x week for 12 weeks with above recommendations for physiotherapy. Copy of recommendations have been sent with [worker] to provide to his treating physiotherapist.
3. If symptoms do not improve or return in the future it is recommended [worker] be reassessed in the Shoulder Specialty Clinic as he does [have] an intrasubstance defect of his supraspinatus tendon seen on MRI.

[19] By report dated September 14, 2015, Dr. Mathew advised Dr. George as the worker's family physician:

[The worker] was seen back at the [employer's] clinic today with regards to his right shoulder. Since the injection in June, he has improved dramatically. He still, however, has pain in the anterior aspect of the shoulder. He has since also seen Dr. Faber who has, I believe, suggested continued physiotherapy. I would recommend the same. At this time, he does continue to have anterior pain. I will order an MRI as was suggested by Dr. Faber as well to investigate.

[20] On referral by Dr. Mathew, the worker underwent a multi sequence MRI of his right shoulder with contrast dye on November 10, 2015. That diagnostic procedure was reported by Dr. R. Popuri, radiologist, to disclose:

Focal almost full thickness tear is seen in the anterior insertional aspect supraspinatus tendon measuring 0.6 x 0.3 cm in axial dimension. There is associated adjacent tendinopathy and possible minor calcification.

[21] On November 30, 2015 Dr. Mathew reported to Dr. George that in his opinion the worker's partial-thickness tear was quite small and Dr. Mathew believed the worker had a good chance of improvement with physiotherapy alone.

[22] On March 21, 2016 Dr. Mathew reported to Dr. George that given the worker's reported flare ups of right shoulder pain despite ongoing physiotherapy, Dr. Mathew was referring the worker for a further MRI to determine whether the worker's subscapularis was also torn and to reinvestigate the supraspinatus tear as observed on the November 10, 2015 MRI.

[23] On April 4, 2016 a Case Manager ("CM") with the Board wrote to the worker and confirmed that the worker's claim for a right arm/shoulder strain arising from the May 28, 2015 compensable accident was approved. However, the CM determined that the subsequent identification in the MRI of November 10, 2015 of an almost full-thickness tear in the worker's right supraspinatus tendon was not compatible with the mechanism of injury occurring on May 28, 2015, and therefore the worker did not have entitlement for that identified right shoulder condition. In part, the CM wrote:

I have reviewed [the worker's] file to assess the results from the MRI of November 10, 2015 and also obtained a medical opinion. There is a link that can be established between the mechanism of the incident you reported and the diagnosis of right shoulder strain. Rotator cuff tears are typically degenerative in nature and are a natural consequence of aging. The mechanism of injury of note is not of substantial enough force to cause a rotator cuff tear as you did not report any sudden or strenuous force through the shoulder, nor is it likely responsible for the minor edema reported in the teres minor muscle which is more consistent with degenerative etiology. As such, the results of the MRI in question from November 10, 2015 are not compatible with the mechanism of injury.

[24] The CM's decision of April 4, 2016 to deny the worker's entitlement to the findings in the November 10, 2015 MRI diagnostic imaging of the worker's right shoulder, relied upon an Initial Physician Case File Review Memo prepared at the request of the Board by Dr. Z. von Aesch, a family medicine practitioner. In that memorandum, Dr. von Aesch described her understanding of the May 28, 2015 mechanism of injury to be; "...the worker reporting pain in his right shoulder as a result of climbing a ladder and putting his full weight on his right shoulder." Dr. von Aesch advised the Board that the described mechanism of injury was not of substantial enough force to cause a rotator cuff tear. Dr. von Aesch concluded her advice to the Board by stating:

The worker had a pre-existing rotator cuff condition as noted both by imaging and report from Dr. Mathew [sic], Orthopedic Surgeon on July 6, 2015 detailing an injury the worker had 6 months prior due to an overhead pull. noting severe shoulder pain ever since. An ultrasound report prior to the DOI in March 2015, noted tendinosis of the supraspinatus tendon. The MRI's following the injury are consistent with the natural progression of the supraspinatus tendinopathy and degenerative change. As such, the results of the MRI in question from November 10, 2015 are not compatible with the mechanism of injury.

[25] On May 16, 2016 Dr. Mathew reported to Dr. George that as the worker's right shoulder was not getting any better, Dr. Mathew would proceed with surgery. Dr. Mathew stated, in part:

I understand that the claim for some reason has been denied with regard to his right shoulder. This gentleman is known to have a rotator cuff tear on his MRI. In his age category, it is extremely uncommon to have a rotator cuff tear and this gentleman does not give any history of previous injury. Therefore, I would opine that the injury is directly related to his repetitive work here at [employer], as there is no other contributing factor.

[26] In advancing the worker's objections to the CM's decision of April 4, 2016 through to an appeal before an ARO, the worker's representative requested by letter dated February 20, 2019 a further opinion from Dr. Mathew concerning the worker's right shoulder injuries. Regarding the issue of entitlement for the November 10, 2015 diagnostic findings, the question asked of Dr. Mathew was:

[The worker] had an accident at work in which he slipped while climbing a ladder and ended up hanging on with his right arm May 28, 2015 [sic]. (Dr. Mathew letter to Dr. George dated July 6, 2015 and MRI dated June 1 2015) [The worker] had a new MRI October 11 2015 [sic] which found an almost full thickness tear in the insertional aspect of the supraspinatus tendon anteriorly.

1. Since the MRI without contrast of June 1 2015 did not show this tear, which was taken 4 days after fall on Arm May 28, 2015 and the MRI Arthrogram, October 11 2015 [sic] did show the tear, does this mean the tear was not the result of the May 28 2015 slip and fall hanging on the Right Arm or is this just the difference in imaging between the 2 MRI's?

Or did the fact of the MRI of June 1 2015 was performed 4 days after fall on the right arm matter in regards to what would be visible and how the condition was defined in the June 1 2015 MRI?

[27] Dr. Mathew, in response to the foregoing question, answered in an opinion letter dated February 28, 2019:

[The worker] had and [sic] non-arthrographic MRI performed at [name of hospital] dated June 1, 2015. This was reported as Marked Tendinosis of the supraspinatus tendon. He later went on to have an MRI arthrogram performed at a separate hospital [name of hospital] on October 11, 2015 [sic] which reported a full thickness tear.

As these are two different tests (and also two different MRI machines), I would not opine that this means the tear was not the result of the May 28, 2015 slip and fall and hanging on the right arm. As the initial MRI did not have injection of Dye, a small full thickness tear can manifest itself and appear as marked tendinosis. The test which would provide higher accuracy would be an MRI arthrogram which is what his second test was. With the addition of Dye into the joint prior to the MRI, this would distend all tissues of the joint, and bursal tissue or scar tissue that could appear on a regular non dye MRI as tendinosis, would appear as a full thickness tear. This very well could have been the case in this patient.

[28] The ARO in the decision dated September 18, 2020, referred to the correct date of the MRI date in question as November 10, 2015. “10/11/2015” is used on the report contained in the Case Record and it is understood by the Panel that this is in “day/month/year” format and not as was described by the worker’s representative or Dr. Mathew as October 11, 2015. The ARO also noted that the worker’s representative described the May 28, 2015 accident as a “slip while climbing a ladder with hanging on the right arm” and Dr. Mathew in turn described the May 28, 2015 accident as a “slip and fall hanging on the right arm.” The ARO concluded:

This is not what was reported by the worker, or accepted in this claim.

Instead, the worker states he did not slip, and that he only felt a shoulder pull when he twisted his body while hopping up the ladder steps using the right arm. For these reasons, I have placed more weight on the MC [Board Medical Consultant] opinion that the November 10, 2015 MRI findings are consistent with the natural progression of the supraspinatus tendinopathy and degenerative changes shown in the ultrasound and MRI results prior to the May 28, 2015 workplace accident. As I accept the opinion as provided by the MC, I find the worker does not have entitlement to the November 10, 2015 MRI findings.

[29] As noted, the worker appeals from the ARO’s decision that he does not have initial entitlement to tendinosis of the right supraspinatus tendon, and does not have entitlement to the November 10, 2015 right shoulder MRI findings.

(iv) Law and policy

[30] Since the worker claimed to be injured in 2015, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[31] An “accident” is defined in section 2(1) to include:

- (a) a wilful and intentional act, not being the act of the worker,
- (b) a chance event occasioned by a physical or natural cause, and
- (c) disablement arising out of and in the course of employment;

[32] General entitlement to benefits is governed by section 13:

13(1) A worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan.

(2) If the accident arises out of the worker’s employment, it is presumed to have occurred in the course of the employment unless the contrary is shown. If it occurs in the course of the worker’s employment, it is presumed to have arisen out of the employment unless the contrary is shown.

[33] The statutory presumption set out in section 13(2) does not apply to an injury by disablement. See, for example, *Decisions No. 268*, 1987 CanLII 1992 (ON WSIAT) and *42/89*, 1989 CanLII 2559 (ON WSIAT).

[34] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*, 1987 CanLII 1996 (ON WSIAT).

[35] The standard of proof in workers’ compensation proceedings is the balance of probabilities. Pursuant to subsection 124(2) of the WSIA, the benefit of the doubt is resolved in

favour of the claimant where it is impracticable to decide an issue because the evidence for and against the issue is approximately equal in weight.

[36] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision # 9, would apply to the subject matter of this appeal:

- Package #241 – Initial Entitlement
- Package #300 – Decision Making/Benefit of Doubt/Merits and Justice

[37] We have considered these policies as necessary in deciding the issues in this appeal.

[38] OPM Document No. 15-02-01, “Definition of an Accident”, describes a chance event as “an identifiable unintended event which causes an injury”, an injury itself is not a chance event. The policy defines a disablement as “a condition that emerges gradually over time” or “an unexpected result of working duties.”

[39] OPM Document No. 11-01-01, “Adjudicative Process”, states that an allowable claim must have five points: an employer, a worker, personal work-related injury, proof of accident, and compatibility of diagnosis to accident history. OPM Document No. 11-01-01 provides the following guidelines for determining proof of accident:

Proof of accident

Decision-makers may consider the following when examining proof of accident,

- Does an accident or disablement situation exist?
- Are there any witnesses?
- Are there discrepancies in the date of accident and the date the worker stopped working?
- Was there any delay in the onset of symptoms or in seeking health care attention?

(v) Testimony

[40] Relevant to the issues in this appeal, and in summary, the worker testified:

- The worker’s occupation with the employer is that of a tool and die maker – maintenance.
- The worker has been in this occupation since 1997 but started his employment with the employer in 2012.
- The worker believes that he first injured his right shoulder in January 2015 while torquing a nut on a molding machine used in the employer’s manufacturing operations. The worker described the torquing process as requiring the use of a torque wrench estimated to be about 5 feet in length and weighing 25 kilograms. The nuts to be torqued are located above the worker’s head.
- In questioning by the employer’s representative, the worker’s attention was drawn to an emergency department hospital record that indicated the worker was seen on January 16, 2015 and reported pain in right shoulder for three months and identified no specific event causing injury. The worker acknowledged that this is what the emergency department recorded.

- The worker's primary role with the employer is to address maintenance issues that result in production breakdowns.
- The worker estimated that he was required to climb a vertical ladder at least twice a shift and did even more climbing on night shift.
- The worker also described maintenance duties that required him to crouch, perform inspections and repairs while lying on his back and using both stairs and ladders to access electrical panels that are situated overhead. The worker stated that his work consists of a lot of low heavy lifting.
- Following the reported onset of right shoulder pain in January 2015, the worker performed light duties for two weeks as accommodated by the employer.
- In questioning by the employer's representative, the worker recalled that he engaged in physiotherapy for his right shoulder for a number of months following January 2015 but had stopped physiotherapy for maybe a month or so prior to May 28, 2015 and had been performing his regular job duties.
- The worker testified that on May 28, 2015 that he was moving up a ladder and holding on with his right hand, lost grip with his left hand in which he was holding a check list and twisted outwards on the ladder. He heard or felt a "ping" sound in his right shoulder. This twisting and holding with most of his body weight held on by his right hand and arm caused significant pain in the worker's right shoulder which he immediately reported. This pain was much more severe than he had previously experienced in his right shoulder.
- In questioning by the employer's representative, the worker testified that the accident occurred on a "silo ladder" and the twisting action resulted in the worker "hanging" from his right hand.
- Initial first aid consisted of an ice pack being applied to the worker's right shoulder at the employer's health centre. The worker was placed on modified duties by the employer as his range of motion in his right shoulder was limited, and he could not reach out or tighten nuts.
- In questioning by the employer's representative, the worker testified that he initially saw his family doctor and she arranged for the MRI that was performed on June 1, 2015. The worker thought this might have actually been arranged before the May 28, 2015 accident.
- The worker's right shoulder did not show improvement after the May 28, 2015 accident and the worker went in July 2015 to see Dr. Mathew, an orthopaedic surgeon who comes to the employer's plant and runs a clinic for employees of the employer.
- Initially the worker was told by Dr. Mathew that he might have a tear in his right shoulder. Dr. Mathew did an injection into the worker's right shoulder.
- The pain came back and the worker went to see Dr. Mathew again at the employer's clinic. At no time did Dr. Mathew suggest to the worker that his right shoulder pain was due to a degenerative condition or was unrelated to his employment duties.
- Dr. Mathew had to perform surgery on the worker's right shoulder and the worker was unable to work for eight or nine months following that surgery.

(vi) Submissions on behalf of the worker

[41] In summary, it was submitted on behalf of the worker that his maintenance duties with the employer made a significant contribution to the worker's right shoulder condition as reported in January 2015. The worker's job duties involved regular strenuous and repetitive work, including overhead work such as setting the torque on molding machine nuts. This submission is supported by Dr. Mathew's opinion. Dr. Mathew is an orthopaedic specialist who regularly attends at the employer's facility and runs a clinic at the employer's facility and was familiar with the worker's job duties. Accordingly the worker should be found entitled for the tendinosis identified in the worker's right shoulder and as claimed in January 2015.

[42] With respect to the compensable accident of May 28, 2015, it was submitted on behalf of the worker that by that date, the worker's right shoulder tendinosis condition had somewhat resolved, and that the subsequently identified tear in the worker's right shoulder is entirely consistent with the mechanics of a worker twisting off a ladder and having to hang his body weight on one hand. Again, Dr. Mathew's opinion, as an orthopaedic specialist familiar with the employer's operations, supports this submission and is to be preferred over that of the Board's medical consultant who has no orthopaedic specialty, never saw the worker, and who relies upon an assumption for her opinion that the May 28, 2015 accident could have no impact on the worker's previously damaged shoulder. The Board's MC failed to consider what force would be required to tear a tendon that was damaged by pre-existing tendinosis.

(vii) Submissions on behalf of the employer

[43] In summary, it was submitted on behalf of the employer that as the worker's claim for tendinosis is for a disablement with gradual onset, there is no application of the statutory presumption. The evidence does not support that the worker's condition of tendinosis as diagnosed arose out of his employment as there was no change in job duties prior to reported onset, the worker's duties as described by him do not meet what the Tribunal has generally accepted as constituting repetitive and strenuous activities, and therefore were not likely to amount to a continuous injuring process. Dr. Mathew by describing the worker's job duties as repetitive and involving overhead work does so without identifying a specific understanding of the worker's regular job duties. Dr. Mathew was simply importing an analysis or descriptor of "repetitive" to justify his conclusion. In the absence of a medical opinion that describes an injuring process based upon an analysis of the worker's actual job duties, Dr. Mathew's opinion should not be given much weight. Dr. Mathew appears to suggest in answering the questions put to him by the worker's representative in February 2019 that the worker already had a tear in January 2015 that was diagnosed as tendinosis on an ultrasound, and he says very little about the May 2015 accident. The reference to the worker's young age of 39 by Dr. Mathew and his conclusion therefore that it must be the worker's job that caused his right shoulder injuries fails to address the requirement, in the absence of the presumption of arising out employment, that there must be an identifiable injuring process to establish entitlement.

[44] With respect to the compensable accident of May 28, 2015, the diagnosis of tendinitis as provided by the worker's family physician is clearly compatible with the accident. However, the continued right shoulder complaints by the worker for some time after that strain injury is more consistent with the progression of the worker's pre-existing tendinosis. There was no lost time for the worker following the May 28, 2015 accident, and this indicates that there was no acute injury or tear arising from that accident that incapacitated the worker or required him to seek

immediate medical attention. The Board's medical consultant correctly identified that rotator cuff tears are typically degenerative and her conclusion that the accident of May 28, 2015 was not of substantial enough force to cause a rotator cuff tear is entirely consistent with the mechanics of the accident based upon the worker's description in which no sudden or strenuous force was introduced through the worker's shoulder as it was not a slip or fall. A causal connection cannot be made between the tear identified in the November 10, 2015 MRI and the May 28, 2015 accident.

(viii) Analysis

(a) Entitlement to Tendinosis of the Right Supraspinatus Tendon under the January 2015 Claim

[45] The Panel finds that the worker does not have initial entitlement to tendinosis of the right supraspinatus tendon under the January 2015 claim for the reasons that follow.

[46] In determining the worker's claim the Panel found it useful to refer to the Tribunal's Discussion Paper entitled *Shoulder Injury and Disability*¹ (the "Paper") found in the Case Record and included in the case materials. The Paper was written by Dr. T.S. Axelrod, orthopaedic surgeon, and Dr. D.E. Axelrod, orthopaedic resident. In that Paper, Drs. Axelrod and Axelrod discuss rotator cuff tendon tears and provide some useful definition of medical terms as follows:

[Rotator Cuff Tendon Tears]

Rotator cuff tendons may tear in response to an injury where the external force is greater than the strength of the tendon. These tears can either be where the entire tendon is torn through or partial. A partial tear, commonly known as a partial thickness tear is one where some fibres of the tendon remain intact, holding the tendon together.

[Tendinopathy]

The term tendinopathy refers to any pathology within the structure of the tendon. This may be inflammation without a tear, a partial thickness tear or a complete full thickness tear.

[Tendinitis and Tendinosis]

Tendinitis and tendinosis are interchangeable terms. Defined as follows:

Tendinitis or tendinosis is considered an inflammation of the tendons of the rotator cuff. It is due to microtears of the fibres of the rotator cuff tendons. No actual tear will be seen on the MRI with tendinitis, rather just increased blood flow and swelling (edema) of the tendon is noted.

...

...tendinitis (tendinosis) can occur after trauma or excessive strain on the rotator cuff tendons both in vocational and avocational activities. Rotator cuff tendons that have suffered from tendinosis are poorer quality tissue and tear much easier than strong unaffected tendons.

¹ The Tribunal's medical discussion papers deal with medical topics which frequently arise in appeals. They are written by independent experts who are recognized in their fields of specialization. The papers are not peer-reviewed publications, but rather, are intended to provide parties and representatives with a broad, general overview of medical topics. A discussion paper is included in the case materials for an appeal when it appears that the paper may provide some relevant background to an issue in dispute. Medical discussion papers are also available on the Tribunal's website and in its Library. A Vice-Chair or Panel is not bound by any information or opinion expressed in a discussion paper, but may consider and rely on the general medical information provided by the paper.

...

Tendinitis is a lesser degree of rotator cuff pathology. This term refers to inflammation of the tendon, also known as tendinopathy. On a microscopic level, one can see microtears of the rotator cuff tendons. However, on the macroscopic level, there is no fraying or tearing of the tendon. This inflammation can cause pain and limitations of motion.

[Strain]

A shoulder “strain” is a generic term that does not reflect any specific pathology. It is often used to describe a soft tissue stretch or contusion (bruise) of some muscles around the shoulder in one of two situations. The first would be when first assessing a patient after a shoulder injury in which no imaging has been performed to assess the status of the bones and soft tissues. The practitioner uses the term “strain” to broadly describe shoulder pain due to a soft tissue injury, yet there is no imaging to make an accurate diagnosis. Findings on physical examination maneuvers may suggest rotator cuff pathology, however the injury may be termed a “strain” until the appropriate imaging is performed.

The second situation is when someone has a shoulder injury and imaging (MRI, U/S) is reported as normal. The pain cannot be explained by a disruption of the rotator cuff or labrum, so in the absence of any specific diagnosis, the term “strain” is applied to the injury. This term is often utilized in initial assessment, though the term alone represents an imprecise and often inaccurate opinion of the injury.

[47] Drs. Axelrod and Axelrod provide commentary in the Paper that the terms tendinitis and tendinosis are interchangeable, as both terms describe inflammation of the tendons, also known as tendinopathy, caused by microtears.

[48] The Panel accepts the worker’s description and his testimony regarding his job duties prior to January 2015. Those duties included climbing ladders at least twice a shift, using a large torque wrench several times a week to tighten nuts and bolts on molding machines and other equipment, at times pushing and pulling heavy carts and parts in order to work on manufacturing equipment. The Panel finds that some of the worker’s job activities on occasion involved overhead work.

[49] Dr. Mathew in his opinion of February 28, 2019 described the worker’s job duties as involving:

...significant pushing, pulling, lifting and overhead work. I would strongly opine that this type of repetitive task would be a significant contributing factor in [the worker’s] shoulder problems.

[50] Given the worker’s testimony and the documentation in the Case Record regarding the worker’s statements when asked about the frequency and possible repetition in his work duties; the Panel does not find Dr. Mathew’s characterization to be accurate. As Dr. Mathew provided the only medical evidence of substance that indicated that there was a causal connection between the worker’s tendinosis as claimed in January 2015, and the worker’s job duties at that time, the Panel finds that there is insufficient medical evidence which supports a conclusion that the worker’s tendinosis in January 2015 had as a significant contributing factor the worker’s job duties. For this reason the Panel finds that the worker does not have initial entitlement to tendinosis of the right supraspinatus tendon in the January 2015 claim.

(b) Entitlement to the findings in the November 10, 2015 MRI as Arising from the May 28, 2015 Accident

[51] The Panel finds that the worker does have entitlement under the May 28, 2015 accident claim for the almost full thickness tear seen in the anterior insertional aspect supraspinatus tendon as identified in the November 10, 2015 MRI of the worker's right shoulder. The Panel's reasons are as follows.

[52] As noted above, the Panel finds that immediately prior to the worker's compensable accident on May 28, 2015, he had a pre-existing condition in his right shoulder that was diagnosed following an ultrasound in March 2015 as tendinosis.

[53] As was previously noted from the Paper, rotator cuff tendons that exhibit tendinosis are of poorer quality and tear much easier than strong unaffected tendons.

[54] The worker's compensable accident on May 28, 2015 was accepted by the Board as resulting in an injury of tendinitis. The Board accepted the mechanics of injury to be that in the course of climbing a ladder on May 28, 2015, at some point due to the worker not holding on with both hands, and there being a resulting twisting moment, the worker had his full weight suspended from his right arm. The immediate experience for the worker was severe pain. This is the accepted description found in the EA's memorandum of June 26, 2015. The EA accepted that the accident caused the worker to be supporting his full weight by his right arm and these mechanics of injury were compatible with a diagnosis of resultant tendinitis.

[55] While the worker was able to carry on with modified work duties following the May 28, 2015 accident, the Panel accepts as accurate the worker's recollection that his right shoulder became a source of symptomatic pain which eventually lead him to being assessed by Dr. Mathew in July at a clinic run by the employer for its employees.

[56] The Panel finds that following the worker's compensable accident on May 28, 2015, the worker's right shoulder in addition to its pre-existing tendinosis, had at a minimum, additional microtears caused by an acute trauma, given Dr. George's report that the worker suffered sprain tendinitis from that accident as accepted by the Board.

[57] The question therefore becomes whether the addition of the tendinitis injury caused by the May 28, 2015 compensable accident to the worker's pre-existing diagnosis of tendinosis made a contribution of significance to the worker's right shoulder condition such that the findings in the November 10, 2015 MRI are a reflection of that additional injury.

[58] The ARO in the September 18, 2020 decision relies in part on a conclusion that is addressed in Drs. Axelrod and Axelrod's Paper. The Paper contains this comment:

The vast majority of rotator cuff tears occur without a specific traumatic event, and are often caused by the progression of more minor pre-existing cuff pathology...

However, the Paper also goes on to discuss the potential for degenerative conditions to be subject to an additional element of trauma or injury that can make a contribution of significance:

Quite often a patient will have clear evidence of pre-existing shoulder pathology such as a rotator cuff tear yet will have been minimally or not at all symptomatic before some acute episode occurs. This episode may be severe such as a fall directly onto the shoulder or less so, with the episode being no more than the development of shoulder pain after doing the regular day to day overhead work of one's normal job. It is not at all clear how or why an abnormal, but pain free shoulder becomes symptomatic after any such events.

Perhaps it is the tearing of the few remaining rotator cuff tendon fibers that are keeping the structure functional that causes the change in symptoms, or other physiologic or non-physiologic factors. Not all symptoms will have a clear organic explanation.

In a similar fashion, a patient may have an asymptomatic arthritic shoulder.

After doing the normal workday duties, or some other overhead, lifting, pushing or pulling activity for some unclear reason, the shoulder begins to hurt. From a medical perspective, this is considered an aggravation of an underlying pre-existing condition. It is very hard to sort out the role of workplace duties in this situation, in the absence of any specific out of the ordinary event. The shoulder may just as easily become symptomatic over a leisurely weekend of doing no specific activities that would aggravate it.

Once the symptoms begin, they often persist for a substantial period of time, and may need additional interventional treatment.

[59] The Panel finds that it can give less weight to Dr. von Aesch's conclusion that the force of the May 28, 2015 accident was not sufficient to cause a rotator cuff tear and that the tear observed in the November 10, 2015 MRI was a natural progression of the worker's tendinosis. Dr. von Aesch did not specifically address or consider what were the consequences of the worker suspending his body weight on his right arm given his already damaged right supraspinatus tendon. In short, Dr. von Aesch did not address whether the May 28, 2015 accident was a specific out of the ordinary event that would have aggravated or altered the course of the worker's pre-existing tendinosis.

[60] Dr. Mathew, as has been noted, is of the opinion that the worker's tendinosis which pre-existed the May 28, 2015 accident was occupationally related and this was the focus of much of his reports and commentary. The ARO placed less weight on Dr. Mathew's opinion regarding what contribution the May 28, 2015 accident made to the worker's right supraspinatus tear because the ARO found that Dr. Mathew's opinion was based upon an inaccurate understanding of the mechanism of injury. In the February 28, 2019 opinion letter Dr. Mathew stated in answer to the worker's representative's question concerning the June 1, 2015 MRI:

...I would not opine that this means the tear was not the result of the May 28, 2015 slip and fall and hanging on the right arm. As the initial MRI did not have injection of Dye, a small full thickness tear can manifest itself and appear as marked tendinosis. The test which would provide higher accuracy would be an MRI arthrogram which is what his second test was. With the addition of Dye into the joint prior to the MRI, this would distend all tissues of the joint, and bursal tissue or scar tissue that could appear on a regular non dye MRI as tendinosis, would appear as a full thickness tear. This very well could have been the case in this patient.

The distinction to be noted is that the worker's May 28, 2015 accident was accepted by the Board as involving a ladder climbing incident which resulted in the worker, as noted in the EA's memorandum of June 26, 2015, having, "his full weight being carried by the right arm causing shoulder pain," and Dr. Mathew describes the incident as, "a slip and fall and hanging on the right arm."

[61] The Panel has not come to a similar conclusion to that of the ARO and finds that weight must be given to Dr. Mathew's opinion concerning the May 28, 2015 accident. Dr. Mathew's opinion was premised on an understanding that the accident of May 28, 2015 resulted in the worker holding his body weight by his right arm and that this accident contributed to the subsequently identified tear. While there is relevancy to understanding the sequence of events that lead to the worker carrying his full weight by the right arm, the Panel finds that whether that

was due to a missed handhold and a resultant twist or a slip is not determinative of the significance, as an injuring process, of the worker carrying his full weight by his right arm. Dr. Mathew's opinion is expressed in an awkward negative in answer to the worker's representative's question about the June MRI result as Dr. Mathew states; "I would not opine that this means that the tear was not the result of the May 28, 2015 slip and fall and hanging on the arm."

[62] The Panel finds that Dr. Mathew, as the worker's attending orthopaedic specialist, has expressed an opinion that identifies the May 28, 2015 accident as a specific out of the ordinary event that altered the course of the worker's pre-existing tendinosis.

[63] Given the Panel's findings, it is left with a decision about whether it has been reliably shown that the almost full thickness tear in the worker's right supraspinatus tendon was solely the consequence of a natural progression of the worker's tendinosis identified in onset prior to May 28, 2015. The Board has determined, based upon Dr. George's report of a trauma injury, that the accident of May 28, 2015 caused a tendinitis injury to the worker's rotator cuff tendons. Given the accepted trauma and the injuring process of a full body weight hang sufficient to cause tendinitis, the Panel finds, based upon the medical opinion of Dr. Mathew, as the worker's attending orthopaedic specialist, and supported by the noted information contained in the Paper, that on a balance of probabilities, the trauma caused to the worker by the May 28, 2015 accident made a contribution of significance to the development of the almost full thickness tear observed in the November 10, 2015 MRI. For these reasons, the Panel finds that the worker has initial entitlement to the November 10, 2015 MRI finding of an almost full thickness tear of the right supraspinatus.

DISPOSITION

[64] The appeal is allowed in part as follows:

1. The worker does not have initial entitlement to tendinosis of the right supraspinatus tendon as a result of a gradual onset disablement under the January 2015 claim.
2. The worker has entitlement under the May 28, 2015 claim for the condition related to the November 10, 2015 MRI diagnostic imaging of the worker's right shoulder which identified an almost full thickness tear of the worker's right supraspinatus tendon.

[65] The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: October 13, 2021

SIGNED: R.E. Salisbury, M. Kesler, S. Roth